MAHATMA GANDHI UNIVERSITY, KOTTAYAM

MGU-UGP (HONOURS) Regular EXAMINATION October 2025

Third SEMESTER

Discipline Specific Core Course (DSC) - MG3DSCPSY202 -

PSYCHOLOGICAL FIRST AID

(2024 ADMISSION ONWARDS)

Duration: 2 Hours **Maximum Marks: 70**

Remember(K), Understand(U), Apply(A), Analyse (An), Evaluate(E), Create(C), Skill(S), Interest(I) and Appreciation(Ap)

Students should attempt at least one question from each course outcome to enhance their overall outcome attainability.

Part A

Multiple Choice Question-
Choose the most appropriate answer
Answer all questions
Each question carries **1** marks

1. What is the key principle behind offering practical information? [K] / [CO1]
   c). To reduce anxiety fueled by uncertainty.

2. Which of the following illustrates the use of PFA? [U] / [CO1]
   b). A volunteer providing reassurance and a sense of safety

3. A key outcome expected after PFA is- [U] / [CO1]

   c). **Improved coping and reduced distress**

4. Which of the following is a long-term objective of PFA? [U] / [CO1]

   a).**Promoting resilience and community recovery**

5. The core competency described as "interventions designed to stop escalating psychological and behavioral reactions" is known as: [K] / [CO1]

   b). **Stabilization**

6. Which of the following best describes acute grief ? [U] / [CO1, CO2]

   b). A painful but natural reaction to loss

7. What factor most strongly contributed to increased PTSD risk among recent Iraq and Afghanistan war veterans? [U] / [CO1, CO2]

   b). Witnessing or being responsible for killing

8. Which factor helps determine whether an emergency qualifies as a disaster?[U] / [CO1, CO2]

   b). Severity of impact, amount of damage, and support needed

9. The Oklahoma City bombing resulted in approximately how much property damage? [U] / [CO1, CO2]

   c**). $625 million**

10. Which disaster led to one of the largest increases in sexual violence reports? [U] / [CO1, CO2]

    b). **Haiti Earthquake**

11. Which of the following should be avoided in PFA? [U] / [CO2, CO3]

    B) Asking too many closed-ended questions

12. Which of the following is **NOT** considered a symptom of MDD? [U] / [CO1, CO2]

    c) Increased self-esteem

13. Panic attacks are classified under: [U] / [CO2, CO3]

    B) Emotional dysfunction

14. A set of biopsychosocial factors can make some people vulnerable in the wake of: [K] / [CO2, CO3]

    c). Disasters

15. If a child is unaccompanied during a disaster, the PSFA strategy is to: [A] / [CO2, CO3]

    c). Link the child to child protection network

16. Which type of professional support is recommended for elderly with psychological concerns? [U] / [CO2, CO3]

b). Mental health professionals

17. A patient shows dizziness, chest pain, and numbness of the arm after prolonged stress. This pattern indicates: [A] / [CO2, CO3]

B) Physiological dysfunction

18. Which behavior indicates dysfunction rather than distress? [K] / [CO2, CO3]

C) Persistent avoidance

19. Which vulnerable group does not include in factor Disability. [U] / [CO2, CO3]

d) Persons in palliative care

20. Cognitive indicia under distress include: [U] / [CO2, CO3]

B) Temporary confusion and inability to concentrate

[1x20 = 20]

Part B

Short Answer type questions

Answer any 10 questions

Each question carries 3 marks

21. Mention any three factors that increase the severity of psychological symptoms for first responders [U] / [CO1, CO2]

Ans: Events involving children, Lack of social support, Sleep disturbances, Prior mental health issues or trauma history, Low self-efficacy (belief in one's ability to cope), Low income or education, Female gender in some studies, Loss of loved ones or injury, Direct exposure or threat to life, Witnessing injury or death, Prolonged displacement or loss of property, Perceived injustice or lack of control, Occupational stress among first responders. (Any three).

22. Define Stress. Discuss the neural axes of stress response. [U] / [CO1, CO2]

Ans: Stress is the nonspecific response of the body to any demand made upon it. It is a "fight or flight" response intended to do—to prepare you to fight or to flee from a life-threatening event. The body's fastest stress response comes from the neural system, which includes the sympathetic nervous system (SNS) and the parasympathetic nervous system (PNS), both part of the autonomic nervous system (ANS). These systems send signals through neurotransmitters to organs, muscles, and the brain. The SNS activates the fight-or-flight response: it increases heart

rate, dilates pupils, diverts blood to large muscles, and releases glucose for energy, while reducing digestion. The PNS activates the rest-and-digest response: it slows the heart rate and reactivates digestion once the threat is gone. However, too much SNS activation relies on primitive brain functions and suppresses higher-order thinking in the front brain. This is called cortical inhibition, where problem-solving and clear thinking are impaired. This mental confusion can further raise heart rate, creating a negative cycle.

23. Define panic disorder. Describe different disease, disorder and symptoms that are associated with it [U] / [CO1, CO2]

Ans: The DSM-5 describes panic disorder as recurrent unexpected (i.e., no obvious cues or triggers) panic attacks. Panic disorder is associated with psychiatric and physical comorbidity, suicidal ideation and attempts, and overall decreased quality of life. Panic disorder was significantly associated with cardiovascular diseases and peripheral vascular disease.

24. What is psychological triage in the context of mental health? [K] / [CO2, CO3]

Ans: Psychological triage in mental health is the rapid assessment and sorting of people affected by a crisis to decide who needs urgent psychological help first. It involves observing emotional and behavioral signs (like crying, shouting, or freezing), identifying specific needs (such as calming or emotional support), and prioritizing care based on severity. The goal is to provide psychological first aid and support in the most effective and timely way, especially during disasters or traumatic events.

25. Give two examples of professionals or agencies to whom a crisis worker might refer an individual for continued care [A] / [CO2, CO3]

Ans: After providing immediate support, a crisis worker may refer individuals to various professionals and agencies for continued care based on their needs. These include psychiatrists for medication and management of severe mental health issues, clinical psychologists or counselors for therapy and trauma recovery, and social workers who assist with emotional and practical needs such as housing, welfare, or employment. Referrals may also be made to psychiatric nurses for follow-up care, community mental health centers and NGOs for ongoing counseling and rehabilitation, and support groups that provide peer encouragement and reduce

isolation. In cases of abuse or neglect, survivors may be linked to women's shelters or child protection services, while faith-based counselors and rehabilitation centers offer additional emotional and physical recovery options. These coordinated referrals ensure that individuals receive comprehensive, continuous, and holistic care addressing their psychological, medical, and social well-being.

26. Describe types of loss [K] / [CO1, CO2]

Ans: Materialistic Losses: These are visible, measurable losses of things that people own or use. Examples: Destruction of houses, roads, vehicles. Loss of crops, livestock, land. Death of earning members (economic loss). Loss of job or business income.

Non-Materialistic Losses: These are intangible, emotional, and social losses that cannot be easily measured. Examples: Grief, trauma, fear, and loss of self-confidence. Separation from family or community. Breakdown of social and cultural practices. Loss of faith, trust, or sense of security.

27. Discuss how the use of mood-altering substances can affect emergency responders after trauma. [U] / [CO1, CO2]

Ans: People use a wide variety of both legal and illegal mood-altering substances in the wake of disaster, trauma, and general adversity. It's a form of self medication designed to lessen the negative impact of the adversity. When PFA providers are working with emergency responders, it's also important to inquire not only about the use of alcohol but about energy drinks. Energy drinks are sympathomimetic stimulants capable of inducing generalized, potentially excessive arousal. Sleep disturbance, anxiety, difficulty concentrating, tachycardia, and arrhythmias are possible consequences of overuse.

28. Differentiate between fear and anxiety [U] / [CO1, CO2]

Ans: Fear is stimulus specific, causing arousal and apprehension—a fear of snakes, for example. Anxiety is generalized apprehension and arousal, free floating and less associated with a highly defined specific stimulus. When the source of danger is obvious, the experienced emotion has been called fear (e.g., "I'm afraid of snakes"). With anxiety, however, we frequently cannot specify clearly what the danger is (e.g., "I'm anxious about my parents' health"). Fear or panic is

a basic emotion that involves activation of the fight-or-flight response of the autonomic nervous system; it occurs in response to imminent danger. Anxiety is a more diffuse blend of emotions that includes high levels of negative affect, worry about possible threat or danger, and the sense of being unable to predict threat or to control it if it occurs.

29. What is supportive communication in PFA? [U] / [CO2, CO3]

Ans: Supportive communication in PFA refers to expressing care and understanding through respectful words, gentle tone, and nonverbal behaviors such as nodding and open body language. It involves being patient, validating the person's feelings, and offering comfort without judgment. This type of communication helps individuals feel heard, valued, and emotionally secure during a crisis.

30. Define PTSD and list out different group which are vulnerable to develop PTSD [U] / [CO1, CO2]

Ans: A disorder that may result when an individual lives through or witnesses an event in which they believe that there is a threat to life or physical integrity and safety and experiences fear, terror, or helplessness. Symptoms typically develop within the first three months of the trauma, need to be present for more than one month, and cause distressed or impaired functioning.1. Military personnel 2. Marine workers 3. Firefighters 4. Police officers 5. Ambulance personnel 6. Rape survivors

31. How does prioritization in the RAPID Model help mitigate distress in a multi-casualty scenario? [A] / [CO2, CO3]

Ans: In a multi-casualty scenario, prioritization in the RAPID Model helps mitigate distress by enabling crisis workers to quickly identify and attend to individuals who are in the greatest psychological need. Using psychological triage, responders assess emotional, behavioral, and cognitive signs—such as panic, aggression, confusion, or dissociation—to determine who requires immediate intervention. This process prevents chaos and ensures that those at the highest risk of self-harm, severe distress, or endangering others receive prompt support. By following a structured hierarchy of urgency, based on factors like severity of trauma, perceived life threat, and prior psychiatric history, responders can stabilize the environment, reduce

emotional overload, and restore safety and order. Ultimately, prioritization ensures efficient use of limited resources and facilitates quicker recovery for individuals and the larger affected community by addressing the most critical needs first.

32. Describe recovery and adaptation needs [K] / [CO1, CO2]

Ans: Recovery needs: are those that help in coping with the loss and pain triggered by any disaster. The healing process might take time and would be different for each individual. Individual, group, and community level practices (cultural practices, rituals, group activities) help in the healing process.

Adaptation needs: are the higher order needs for any disaster survivor. Building psychosocial competencies among disaster survivors would yield post-traumatic growth – adaptation, coping and resilience.

33. Explain the difference between spiritual distress and dysfunction. [U] / [CO2, CO3]

Ans: Spiritual distress refers to temporary questioning or struggle with one's beliefs, often triggered by challenging life events. Examples include questioning faith or questioning God's actions during a crisis. These experiences are usually reflective and do not stop the individual from engaging in faith-related practices or rituals.

Spiritual dysfunction occurs when struggles with faith become persistent and impair spiritual functioning. Examples include cessation of faith-related practices or projecting one's faith onto others in a way that disrupts relationships or community participation. Dysfunction requires intervention, guidance, or counseling to help the person restore balance in their spiritual life.

[3x10 = 30]

Part C

Essay type questions

Answer any 2 questions

Each question carries 10 marks

34. The RAPID model is often used in Psychological First Aid. How would you use each step (Rapport, Assessment, Prioritization, Intervention, and Disposition) if you were helping someone after sexual abuse? [A] / [CO4, CO5]

Ans: In applying the RAPID model of Psychological First Aid after sexual abuse, the process starts with Rapport,Rapport, where the helper offers empathy, active listening, and reassurance, creating a safe, nonjudgmental space and validating the survivor by saying things like "I believe you." In Assessment, the helper gently checks the survivor's emotional and physical state, ensuring their immediate safety and identifying any need for medical care or signs of severe distress or self-harm. Prioritization then helps determine what needs to be addressed first—such as physical injury, safety concerns, or high emotional distress. During Intervention, the focus is on calming and supporting the survivor through grounding techniques, normalizing their reactions, and offering practical help that restores a sense of control. Finally, in Disposition, the helper connects the survivor with ongoing support such as medical services, legal aid, counseling, or trusted family members, and encourages follow-up to ensure continued care and recovery.

35. Evaluate the Essential Psychological Skills Required for Effective Psychological First Aid. [E] / [CO4, CO5]

Ans: Effective Psychological First Aid (PFA) requires a set of essential psychological skills that enable helpers to support individuals in crisis with sensitivity, empathy, and professionalism. One key skill is active and reflective listening, which helps the survivor feel heard, respected, and understood without judgment or pressure to disclose more than they wish. Empathy is crucial for conveying compassion and building trust, allowing the helper to connect emotionally and validate the person's experiences. Observation and assessment skills enable the helper to recognize verbal and nonverbal signs of distress, identify immediate safety needs, and evaluate the risk of harm or further trauma. Calm presence and emotional regulation are also vital, as the helper must maintain composure and provide a sense of stability to the distressed individual. Additionally, communication skills—using gentle tone, reassuring words, and culturally sensitive language—promote comfort and trust. Problem-solving and decision-making skills help in identifying priorities and linking survivors to appropriate medical, legal, and psychosocial resources. Finally, empowerment and advocacy skills ensure that individuals regain a sense of control, dignity, and hope. Together, these psychological skills form the foundation of effective PFA, ensuring safety, emotional stabilization, and a pathway toward long-term recovery.

36. If you were the crisis interventionist in Matt's role, how would you apply the appraisal step to Claire's case and prioritize her needs after the flood? [A] / [CO2, CO3]

Ans: If I were in Matt's role, I would begin with appraisal to assess the severity of Claire's distress. She shows physiological (fatigue, disturbed sleep), cognitive (racing thoughts, nightmares), and emotional (sadness, helplessness) indicia, suggesting distress but not dysfunction. My first priority would be meeting her physical and safety needs by encouraging rest, food, and hydration. Next, I would support her emotionally through reflective listening and reassurance that her reactions are normal after trauma. I would then assess her cognitive symptoms to ensure they don't progress into dysfunction. Finally, I would strengthen her social support system, encouraging connection with her caring neighbors to help her regain safety, stability, and control.

37. Discuss the impact and consequences of being a caregiver during disaster situations. [A] / [CO2, CO3]

Ans: Caregivers are individuals who provide emotional, psychological, or physical support to people affected by disasters, trauma, or crises. They may include healthcare workers, counselors, volunteers, emergency responders, or family members who help survivors cope with distress and loss. During disaster situations, caregivers play a crucial role in restoring safety, providing Psychological First Aid (PFA), and linking survivors to necessary resources.

Psychological and physical reactions such as irritability, feeling low, lack of energy, problem in sleep and appetite etc.

- Neglecting physical, emotional, and spiritual needs l Inability to find time for self
- Feeling of being unacknowledged l Inability to focus on expected roles
- Inability to be empathetic
- Decreased sense of purpose in life
- Decreased satisfaction in doing PSFA
- Diminished quality of PSFAservices
- Interpersonal issues with the team and family
- Sense of being trapped
- Increased substance use (cigarette, tobacco etc.)

- Over drinking of coffee/ tea
- Social withdrawal **[10x2 = 20]**